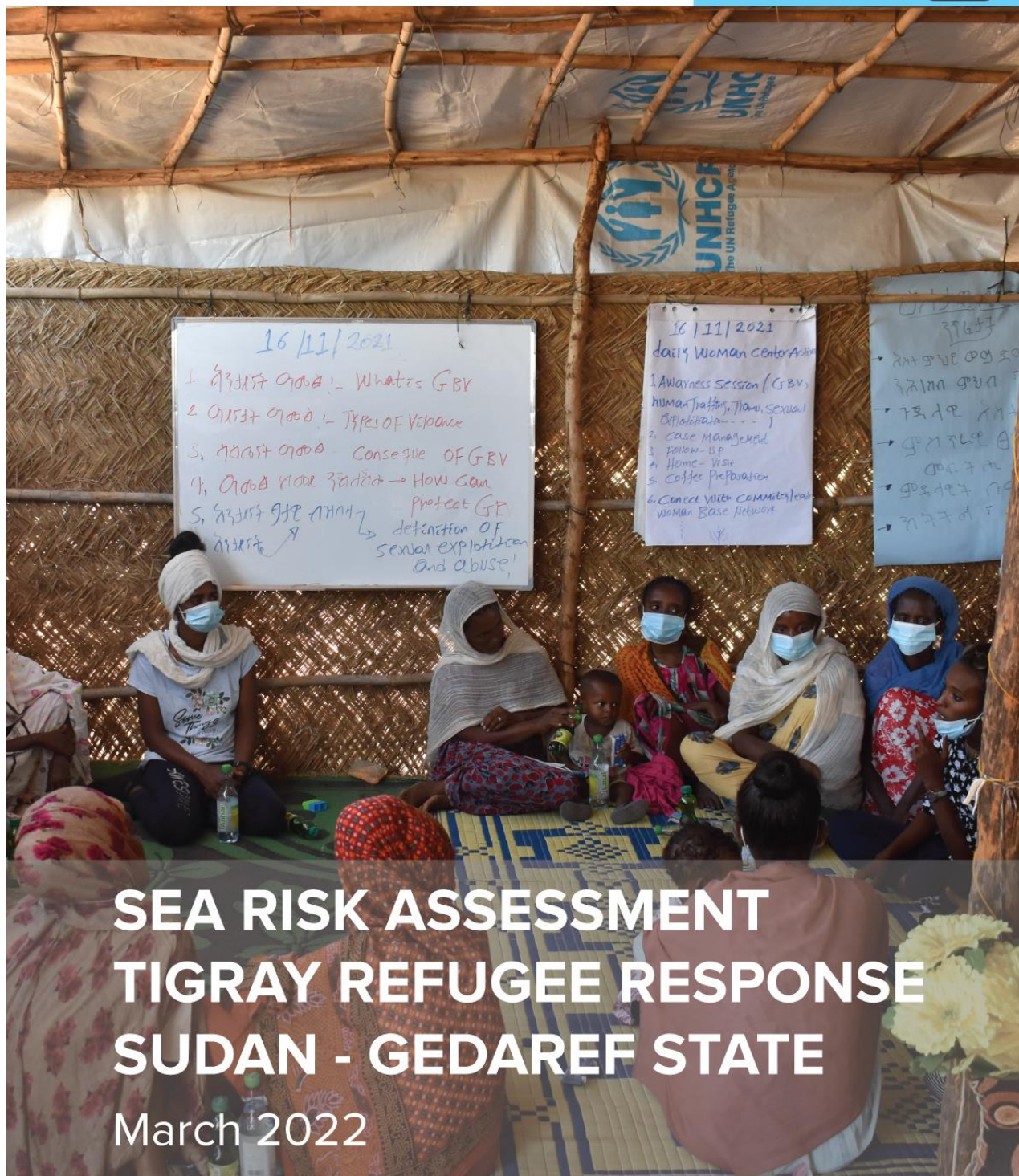
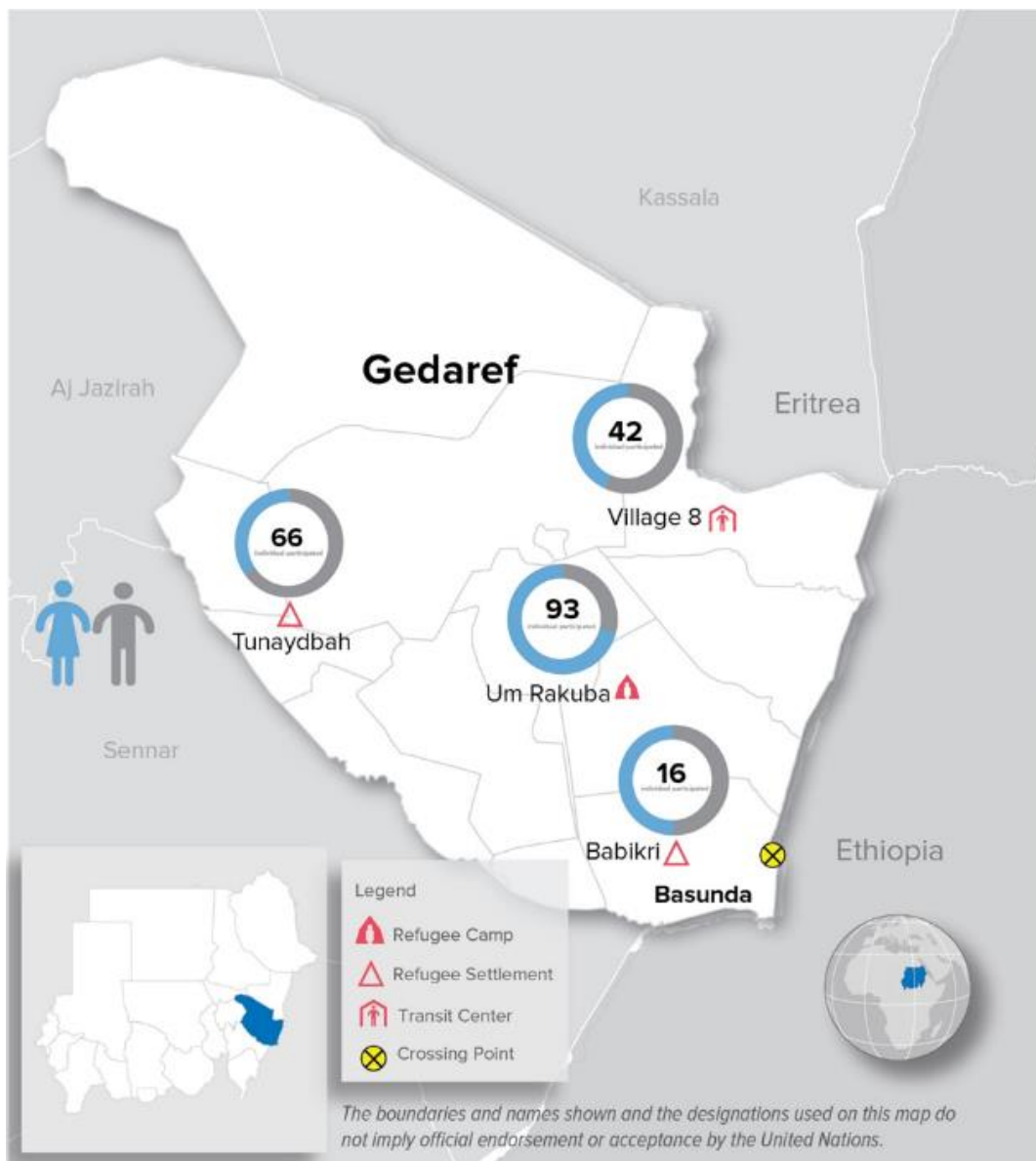




SEA RISK ASSESSMENT TIGRAY REFUGEE RESPONSE SUDAN - GEDAREF STATE

March 2022



217 
People Participated

4 
Locations Reached

9
Key Informant Interviews

17
Focus Group Discussions

8
Activity Monitoring



Mental Health

Most Significant Risks in Terms of SEA for The Community



In-person

The Preferable Reporting Channed for Refugee



Community sensitization (PSEA)

Most Suggested as additional Action to Mitigate SEA



Unclear Complaint Mechanism

Most Answered for Where to Report SEA

1. Background

From the outset of the emergency in Gedaref State, humanitarian partners worked together to prevent, mitigate, and respond to sexual exploitation and abuse (SEA) as a critical element of the refugee response. The risk of SEA inherent in refugee settings, where refugees are in an underlying situation of vulnerability, has been compounded by the absence of functioning Prevention of SEA (PSEA) mechanisms in the State before the onset of the crisis in Tigray when there was limited to no humanitarian presence in the State. In addition, a series of occurrences heightened vulnerability, including harsh weather destroying shelters, interruptions of food distributions for two months, and the remoteness of refugee-hosting locations.

The Refugee Coordination Forum and partners have a zero-tolerance policy on SEA. A PSEA Working Group (WG) was formed in Gedaref at the onset of the emergency and became fully operational in August 2021. Its main objective is to ensure all partners share the same understanding of the risks of SEA identified in the refugee camps and are fully informed to enable a unified response. A set of PSEA training sessions have been conducted and PSEA focal points for all organizations nominated.

To inform PSEA response and develop an action plan with concrete deliverables, UNHCR and partners conducted a PSEA risk assessment in several refugee locations in Gedaref State to identify risks and gaps and to inform the drafting of the PSEA standard operating procedures (SOPs) and action plan.

2. Objectives of the assessment

- 1) Identify SEA risk factors and their drivers at organizational, community, and program implementation levels.
- 2) Review how such risks are being assessed, managed, and/or responded to within the humanitarian response.
- 3) Present recommendations on measures to ensure PSEA safeguarding standards.

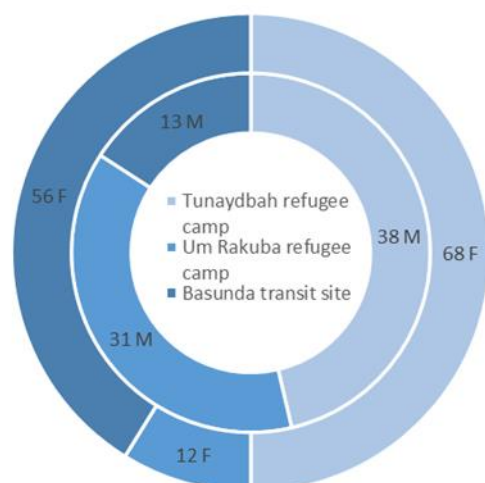
3. Methodology

The SEA risk assessment was conducted using a mix of qualitative methods and direct observations. It should be stressed that the assessment aimed to identify the perception of risk factors and not the prevalence of incidents, which requires a different approach.

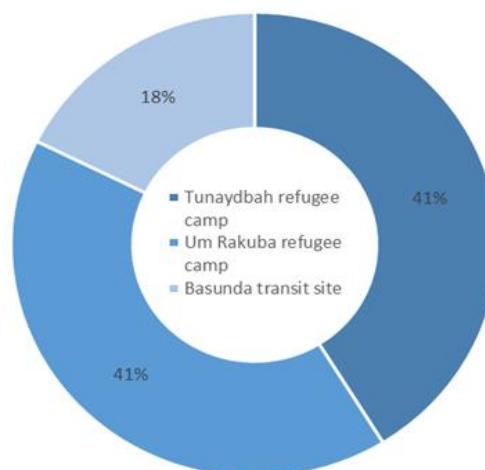
- **SEA assessment tool** (see annex 1) was developed to guide the process, and focused on three key issues:
 1. SEA risks and consequences, according to the perception of the community.
 2. Mitigation measures.
 3. Reporting mechanism, reporting channels, access, and barriers.

- **Ethical safeguards** were integrated into the assessment process including providing clarity on the purposes of the assessment and informing participants of the confidentiality.
- **Key informants' interviews** (KIIs) were conducted with nine (9) individuals including refugee and religious leaders, Commissioner for Refugees (COR) representatives, and Non-governmental Organizations (NGOs) volunteers. For the analysis of the data, overall findings from the KIIs are considered under one group.
- **Focus group discussions** (FGDs) were held with 208 individuals, including women, girls, men, and boys from age groups of 15 to 17, 18 to 24, 25 to 39, and 40 years and above, forming a total of 17 **FGDs**. A specific group was dedicated to Persons with Disabilities (PWDs).

of males (M) and females (F) in FGD per camp



FGD per camp (%)



- **Observations of service points** were conducted in Um Rakuba and Tunaydbah refugee camps in addition to Basunda Transit Center for two days. Facilitators used the agreed data collection tool with a list of points to be verified at each site including safety, visibility, WASH facilities, and medical services.

Observed Locations	Protection Desk	Ore Relief items (CRI) Distribution point	Child-friendly Spaces	COR Office	Ready Meal centers	Primary Health Centers
Tunaydbah	✓	✓	✓	✓	✓	
Um Rakuba						✓
Basunda		✓				✓

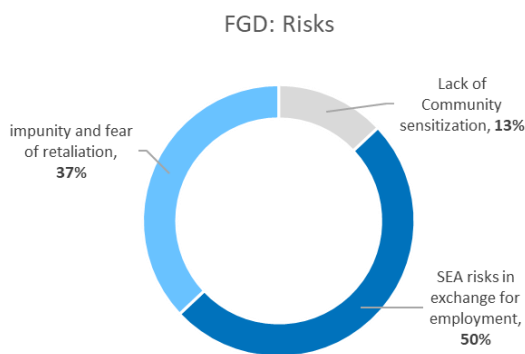
- Data was collected by UNHCR, ALIGHT, DRC, NCA, UNICEF using the KOBO platform. The data analysis is based on the findings from FGDs and KIIs and a desk review of relevant internal reports.

4. Key findings

A. Refugee vulnerabilities in terms of SEA risks and consequences

Risks

- Fifty (50) percent** of FGD participants perceived **SEA risks in exchange for employment, goods, and services**. The following factors were highlighted:
 - Recruitment procedures for community volunteers and incentive workers are not seen to be transparent**, and there is a perception that the few available opportunities are being monopolized by a selected group of people- this includes refugee community leaders, community volunteers, and/or humanitarian partner staff.
 - The distribution process for food and non-food items (including shelter materials, obtaining collection tokens) is perceived to be unclear**, and as a result, it is felt that not all refugees are aware of their entitlements.
- Thirty-seven (37) percent** stated that the perception of perpetrators' impunity, as well as **fear of retaliation**, constituted a potential SEA risk.
- Thirteen (13) percent** identified a **limited community sensitization on refugee entitlements** of services and goods distributed and/or available within the camps, as a risk factor.
 - Women and girls are perceived as particularly at risk of sexual exploitation** during prioritized/targeted distribution and when seeking livelihood opportunities outside the camps.
 - KII stated refugees' **economic vulnerability, lack of livelihood opportunities, limited access to education, transactional sex, and alcohol consumption** as perceived SEA risk factors.



Consequences

- Fifty (50) percent** of interviewees stated **mental health as one of the main perceived consequences** faced by SEA survivors. **Forty (40) percent** highlighted the risks of **unwanted pregnancies and sexually transmitted infections**.

- **Ten (10) percent** mentioned possible **adoption of negative coping mechanisms, particularly alcohol consumption.**

Mitigation measures

- **Thirty-two (32) percent** of FGD interviewees recommended continuous **community-based sensitization** on PSEA as an important factor to mitigate perceived risks.
- **Sixteen (16) percent** of the respondents recommended a **need for greater transparency on the recruitment** process for community volunteers and incentive workers.
- **Eleven (11) percent** recommended community **sensitization on referral pathways available in the camps.**
- **Eleven (11) percent** recommended a **need for stronger protection monitoring.**
- Ten (10) percent recommended the **need for setting clear criteria for the distribution of goods and services.**
- **The remaining 20 percent proposed the following measures:** continuous community awareness on SEA; continuous training for humanitarian staff; improving safety and security within the camps; sensitizing the community on recruitment processes for community volunteers and incentive workers; sensitization refugees on complaints systems and process; increasing humanitarian assistance and sensitization on entitlements; increase protection monitoring; access to education; the increased presence of humanitarian workers and lastly provision of cooking fuel.

B. Reporting channels, modalities, obstacles, and mitigation measures

Reporting channels

- **Twenty-four (24) percent** stated that, to their knowledge, SEA cases are **reported to local police.**
- **Twelve (12) percent** stated SEA cases are **reported to health service providers.**
- **Twelve (12) percent** stated SEA cases are **reported to partners, particularly through women centers** in the camps.
- **Six (6) percent** mentioned **reported SEA cases via protection hotlines.**
- Thirty-five (35) percent of participants said they were not aware of the SEA **complaint mechanism in terms of where to report and the process.**
- **Eleven (11) percent did not respond to the question.**

Reporting modalities

- **Twenty (20) percent** of the interviewees would prefer to **report SEA cases to service providers in person.**

- **Seventeen (17) percent** would prefer to **report through community leaders.**
- **Thirteen (13) percent** would prefer to **report via hotlines.**
- **Nine (9) percent** would prefer to **report through protection desks, UNHCR, and to local police.**
- **Six (6) percent** would prefer to report through organizations providing protection services in the camp.
- **Thirty-five (35) percent stated they are not aware of the SEA complaint reporting mechanisms and processes.**

Obstacles to reporting

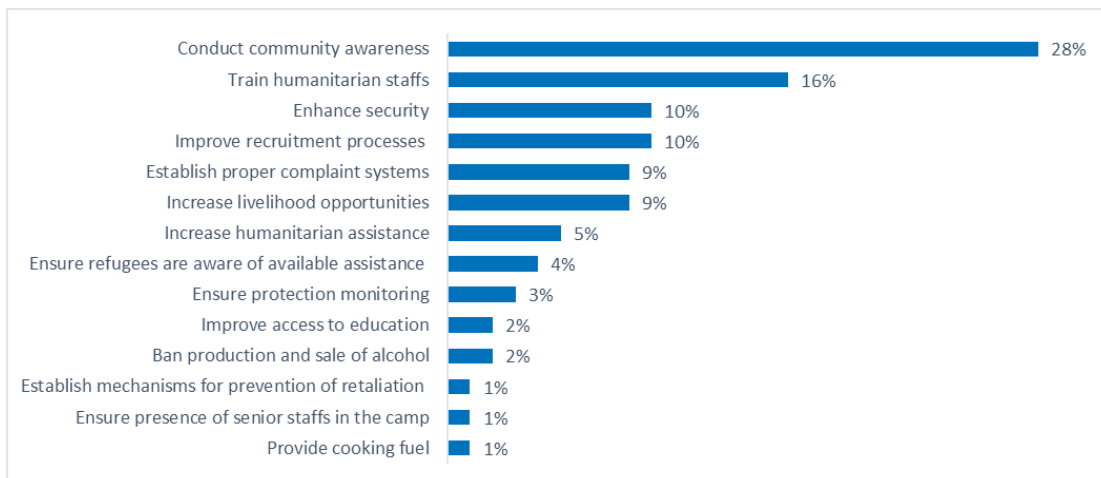
- **Thirty-five (35) percent** of the participants stated they are **not aware of the SEA complaint reporting mechanism and processes.**
- **Twenty (20) percent stated that fear of retaliation** and safety concerns could deter reporting.
- **Fifteen (15) percent** perceived the existing reporting **mechanism as having limited confidentiality measures.**
- **Nine (9) percent** said they believed that refugees are **not aware of the principles of SEA.**
- The remaining **21 percent of the responders** mentioned the following **other factors as perceived obstacles to reporting:**
 - **Language barriers between refugees and service providers were mentioned.**
 - **Lack of trust in reporting SEA through suggestion boxes.** The respondents particularly mentioned the procedures for opening suggestion boxes and locations of the boxes are perceived as not confidential.
 - Protection **hotlines believed to be not free of charge – even when they in fact are -**, and with no-Tigrinya speaker responders.
 - **There is a perception that feedback on action taken on SEA cases are not provided to complainants and SEA survivors.**
 - **There is a perception that legal action is not taken for reported SEA cases.** Other reported issues related to access to justice include limited access to the police stations to report cases given their distance from camps and lack of transport money.
 - **Refugees feel that the number of staff operating protection desks** is limited, causing refugees to wait a long time before they are attended.
 - There is a perception among refugees that disciplinary actions are not taken against perpetrators of SEA when reported.
 - Refugees feel that some service providers display a negative attitude towards refugees when they report SEA and SGBV cases. The assessment did not record the nature and type of 'bad attitude'.

Mitigation measures

- Respondents suggested the implementation of measures that will make reporting SEA simpler and more accessible.
 - **Additional PSEA posters in local languages.**
 - **Increase awareness-raising** through various channels such as the local radio.
 - **Involve law enforcement officials in awareness-raising.**
 - **Involve other refugee community stakeholders** such as youths to carry out SEA awareness sessions, in addition, to **engaging host community** members in SEA awareness sessions.
 - Increase the **number of female police officers** and female staff as service providers to facilitate reporting from female refugees.

Other mitigation measures on wider protection issues were also discussed:

- Increase security patrols to enhance safety in the camps.
 - Ensure Post-Exposure Prophylaxis (PEP) Kits are available.
 - Provide condoms to prevent HIV infections.
 - Service providers should prioritize refugees with special needs at protection desks and food distribution sites.
- Triangulation of the KII and FGD datasets to determine how frequently a proposed mitigation measure was mentioned in terms of percentages is indicated in the table below.



C. SEA Referral pathway and assistance

- **Thirty-three (33) percent** of respondents mentioned **clinics** managed by the government and partners.

- **Twenty-one (21) percent** mentioned that **psychosocial support** can be provided at women's centers and protection desks.
- **Eleven (11) percent** mentioned that **protection service providers** could respond to SEA cases.
- **Thirty-five (35) percent** stated they **did not know where they would go** to seek assistance on SEA cases

Perceived challenges in the referral pathways

- Limited knowledge of available services within the camps.
- Perceptions of limited confidentiality by some service providers lead to ***fear of retaliation*** and social stigma if the information is leaked.

D. General observation

Site observations were conducted at protection desks, Core Relief Items (CRI) distribution points, child-friendly spaces, COR office, ready meal centers, and primary health centers in Basunda transit center, Tunaydbah, and UmRakuba camps. The following was observed:

- **Safety and accessibility:** All locations are safe and accessible, however, there is a need to ensure waiting areas at CRI distribution points and protection desks are shaded. Security guards from refugees and host communities are present in all locations. However, no female security guards were seen on duty.
- **Gender segregation:** All locations have separate queues for males and females at distribution points, with exception of Primary Health Care Centers (PHC) where males and females join the same line.
- **Female staff:** There are a limited number of female staff members among all partners.

Staff presence: Service providers staff and community volunteers are present in all locations and wear visibility materials.

- **Complaints and Feedback Mechanism:** complaints boxes are present in all locations. There is a need to ensure suggestion boxes are visible and located in areas that safeguard confidentiality. Need to install additional visibility materials, particularly explaining all services are free of charge.

5. Analysis

- Fifty (50) percent of the respondents noted SEA risks related to employment and distribution of goods and highlighted the poor economic status of refugees possibly resulting in negative coping strategies and increasing risks. There is a perception that transactional and survival sex is used to gain income, given limited options for securing livelihoods through other means. Limited access to education, and lack of awareness on free entitlements available, was also noted as a perceived risk factor.
- The assessment highlighted several issues that need follow-up, including the need for efficient vetting of community volunteers and incentive workers to mitigate the risks of SEA and so that persons

responsible for SEA or sexual misconduct, are not able to be recruited by different organisations. The respondents also pointed out perceived challenges in the investigative and disciplinary process, highlighting the need for better communication on the processes within the limit of confidentiality.

- Refugees perceive that most SEA risks arise in the process of providing goods and services. This highlights the need to mainstream protection risk analysis across all sectors.
- The respondent stressed the need to make reporting mechanisms known, accessible, and trusted. Face-to-face reporting was highlighted as the preferred mode of reporting SEA, implying a certain level of trust in the protection response mechanisms in place. Fifteen (15) percent of the respondents indicated concerns about the confidentiality nature of SEA reporting mechanisms. Fear of retaliation was specifically mentioned, highlighting a need for appropriate measures to ensure the safety of survivors of SEA and those who report SEA cases.
- The respondents mentioned perceived challenges in the SEA referral pathways, calling for mitigation measures that will enhance SEA survivors having access to appropriate support, including medical, psychosocial, and legal assistance.
- All groups noted the need to provide training to SEA responders and refugee communities on PSEA's key principles, reporting mechanisms, and modalities to enable systematic identification and prevention of risks.
- There is a widespread perception that some profiles among the refugees are more vulnerable to SEA, including women and children and people with mental health. This calls for the increased data collection on vulnerability profiles to build a more informed picture and allow stakeholders to provide an evidence-based response.
- SEA preventive action must consider wider social welfare issues and appropriate support to survivors. This includes providing increased access to livelihood and education opportunities, implementing programs targeting the youths, and strengthening accountability to the affected populations by ensuring effective communication channels and community participation.

6. Recommendation

The assessment provides insight into perceived SEA risks among the refugees, as well as perceptions on Complaints and Feedback Mechanisms, available assistance, and obstacles to reporting. While limited in scope, the assessment provides a baseline for strengthening the response and developing more in-depth assessments of risks patterns. It also points out the way towards which protection factors and processes are perceived as efficient in mitigating risks.

Drawing from the mitigation measures proposed by the respondents, specific action should be taken to strengthen the already established mechanisms for preventing and responding to SEA, ensure compliance with SEA safeguarding standards, and improve protection systems. The interplay of power, control, and social inequalities between refugees and potential perpetrators of SEA needs to take into consideration different levels of risks – i.e., individuals, households, communities, and institutions.

The following table proposes a range of action points that could progressively improve PSEA outcomes from 2022 onwards.

Management and Coordination

- **Review and strengthen as needed safeguard measures during the process** | of community volunteers and/or incentive workers. Procedures should be transparent and known, with clear information shared on recruitment criteria, timelines, and background checks. Partners maintain a joint database of community volunteers and incentive workers that have been vetted.
- **Expand Protection and PSEA training to local authorities** including the police, military, COR, and immigration staff, and continuous refresher training for humanitarian personnel working in direct contact with refugees and the host community.
- **Fortifying inter-agency coordination** through PSEA Working Group and implementation of the PSEA Standard Operating Procedures and Action Plan for Gadaref State.
- **Advocacy on increased safety and security in the camps** through protection monitoring, increased female law enforcement officers, and establishment of community safety networks in coordination with COR and local police.

Accountability to Affected populations

- **Strengthen outreach to communities and community-based feedback mechanisms (CBCM)** to facilitate and improve SEA reporting and community engagement. Prioritize in-person reporting as the preferred modality by refugees.
- **Increase information, education, and communications**, in consultation with refugees, on 1) prevention of and response to SEA (including referral pathways), 2) complaint mechanisms and their process, 3) entitlements at all distribution sites.
- **Identify community-based PSEA focal points**, especially women community leaders and religious leaders, and engagement through alternative media channels such as radio and drama.
- **Promote the participation of female refugees** in leadership committees, particularly those dealing with the distribution of goods.

Prevention

- Regular **capacity building and awareness-raising activities**, to enable relevant stakeholders' staff to identify and understand the impact of SEA and to efficiently take up their role in preventing and responding to such misconduct.
- Mass **community sensitization** Tigrinya, Amharic, and Arabic languages on available **referral pathways in all locations**,
- Address the **fear of retaliation** concern through the regular engagement of senior staff of each organization in discussions on the topics with staff and refugees
- **Sensitize both refugees and the host community on legal proceedings**, ensuring that all are aware of their rights and responsibilities according to Sudanese laws.

Response

- **Enhance processes and procedures for the handling of SEA cases** to ensure a systematic, coordinated, and victim-centered approach. Ensure the survivors are kept abreast of developments when cases are reported.
- **Prioritize access to safe and accessible services for SEA survivors** through established gender-based violence and child protection programming, and advocate

for these programs to be adequately funded. PSEA WG to share regular guidance to ensure all activities encompass PSEA.

- **Carry out analysis of service providers' response to SEA cases** to create an evidence base for the identification of good practices as well as areas needing improvement in coordination with the SGBV WG.
 - When there is a breach of conduct and reporting done, ensure that **investigation is conducted per standards and disciplinary measures** applied against the perpetrator
-

7. Limitations

This assessment was limited by the small sample size, particularly for Key Informants. A total of 208 people participated in focus group discussions and nine (9) people participated in key informant discussions, making a total of 217 respondents in three locations with the following populations as of 1 February 2022; Tunaydbah refugee camp 23,617 individuals; Um Rakuba refugee camp 19,073 individuals; Village 8 transit center 3,143 individuals.

Consolidation of responses from FGDs would have benefited a more detailed recording of information to enhance understanding of critical issues mentioned and cross-verification of responses on similar issues from different groups. This could be attributed to various factors including translations from local languages to English and the ability of different interviewers to probe key issues.

The assessment responses did not comprehensively address all planned objectives as the focus was mainly at the community levels. Very limited information was provided at organizational and program implementation levels.

Nonetheless, given the zero-tolerance standard on SEA, respondents were able to identify key issues and concerns on SEA which are presented in the assessment report.

Annex 1

Data Tool 1: Key Informant Interviews

Name of Key Informant/s: _____ # of Participants: M / F

Location of Interview (Camp/Transit Site/Block): ---- _____

Getting Started and Topic Introduction:	- Thank the key informant for having agreed to participate in the FDG and having welcomed you into his/her community. - Identify yourself. - Ask the key informant to identify him/herself. - Explain that you are going to ask him/her some questions and hope to hear his/her thoughts, opinions, and views about the sensitive topic, as well as the context more generally. This is likely to take about half an hour. - Explain to the participants that the topic of discussion is sensitive as it involves the behavior, conduct, and integrity of humanitarian workers or agency staff. (All those involved in the humanitarian response) - Invite the key informant to be as open as possible when discussing the topic, as his/her responses will contribute to safe programming, that upholds the rights and dignity of persons of concern and ensures that they are not subjected to sexual exploitation and abuse by humanitarian workers. - Clarify that you will be taking notes, but that you are not keeping any written note of personal data, e.g., names of participants. This is an anonymous exercise. - Confirm that they give their consent to this information being recorded and shared, in general terms, within the humanitarian community, and with key stakeholders
Key Informant Briefing:	Highlight the below to participants (in a clear, open, and non-threatening tone): - Sexual Exploitation and abuse constitute a form of sexual misconduct perpetrated by UN/INGO/NGO and affiliated staff, volunteers, and contractors against community members receiving assistance and protection. Examples of such prohibited behavior include: - Engaging in any sexual activity with minors or persons below 18 years – This includes grooming or betrothal to minors awaiting them to turn 18yrs. - Initiating or participating in a sexual or intimate relationship with persons receiving assistance and protection, - Exchanging goods, services, employment, or money for sex or sexual favors from beneficiaries or program participants, - Making any form of sexual advances towards beneficiaries or program participants,

	• Having sex with beneficiaries or program participants forcibly or using threats. • Sexual exploitation and abuse of persons receiving assistance and protection is a form of gross misconduct, and therefore grounds for termination of any implicated staff member. Additional measures include referral for criminal proceedings as per national laws. • The humanitarian community has a responsibility to protect all people receiving humanitarian assistance and protection from sexual exploitation and abuse, by ensuring that: • safe hiring practices are in place • all staff sign codes of conduct • all staff are trained and sensitized on the standards of conduct and SEA • All persons of concern are treated with dignity and respect • All persons of concern can safely report any incidents/allegations/concerns/suspicions of SEA against them or other community members and receive immediate medical, psychosocial, material, and protection assistance. • All information you share will help us to identify additional gaps or risks of SEA that exist and enhance measures to prevent and respond to any SEA occurrences. • Ask if they have any questions about the process, before proceeding.
CONVERSATION GUIDE:	
Question 1: Risks	What, in your view, are the most significant risks in terms of SEA for this community?
Question 2: Mitigation	In addition to the measures outlined in the briefing, are there additional actions the humanitarian community/response could take to better protect persons of concern from these risks? If so, what more can be done?
Question 3: Reporting	In terms of available reporting mechanisms for SEA: • Do refugees know where and how would they report such incidents? • What reporting channels would refugees normally prefer to use? • Are there any obstacles to reporting? • What measures could be put in place to make reporting easier?
Question 4: Services	In terms of assistance for survivors of SEA: -Do refugees know where they can go for assistance following an incident of SEA? -Where would refugees prefer to go? (Prompt – medical, psychosocial, and legal support) -Are there any obstacles to accessing assistance? -What could the humanitarian community do to make it easier to access assistance?
Final Remarks:	Highlight the below to participants (in a clear, open, and non-judgmental tone):

	Importance of Reporting: 1. If the key informant receives a report of SEA, he/she should help the victim/witness to report confidentially to the concerned agency for action. 2. Inform the victim that medical, psychosocial, material, and protection support are available for all victims of SEA, only if the victim is willing to receive such services. Investigations will be conducted, but the victim may decide whether to participate and whether to pursue remedial/reparative action (he/she will not be forced) Advise the complainant, victim, or witness to report through: 1. Protection Desks 2. WFP/UNHCR Hotline 3. A trusted agency office/focal point 4. A trusted community member 5. A trusted agency staff member You can guide them to a trusted focal point or reporting channel and help them make a report while being considerate, respectful, and empathetic. Assure him/her that all reports will be processed and handled with confidentiality, for their safety. The concerned agency will immediately link the victim or witness with relevant service providers, and initiate investigations if they are required to do so.
Conclusion:	• Ask the key informant if they have any further comments or questions. • Thank him/her for their contribution to the conversation and service to the community.

Data Tool 2: Community Level Focus Group Discussions (at least 3 FGDs per location)

Group Category: _____ # of Participants: M / F
Location of Interview (Locality/Settlement/Block): ---- _____

Preparations:	• familiarize yourself with the questions by reading this guidance well in advance • Find a safe and private location for the FGD. • Separate Men, women, youth, and children and if possible, PWDs and carry out separate FGD for each. • Politely ask the community leaders at the location to step aside and allow you to speak to the participants on your own. • Each group not to exceed 15 members
Getting Started:	• ensure that everybody is sitting comfortably and is visible • Thank the participants for having agreed to participate in the FDG and having welcomed you into their community. • Identify yourself. • Ask the participants to identify themselves. • Explain that you are going to ask them some questions and hope to hear their thoughts, opinions, and views about the sensitive topic, as well as the context more generally. This is likely to take about 1 hour. • State that everybody's opinions and experiences are valid, irrespective of who they are. Encourage the group to respect each other's opinions. • Ask if they have any questions about the process, before proceeding.
Topic Introduction:	• Explain to the participants that the topic of discussion is sensitive as it involves the behavior, conduct, and integrity of humanitarian workers or agency staff. (All those involved in the WD humanitarian response)

	• Invite the participants to be as open as possible when discussing the topic, as their responses will contribute to safe programming, that upholds the rights and dignity of persons of concern and ensures that they are not subjected to sexual exploitation and abuse by humanitarian workers. • Clarify that you will be taking notes, but that you are not keeping any written note of personal data, e.g., names of participants. This is an anonymous exercise. • Confirm that they give their consent to this information being recorded and shared, in general terms, within the humanitarian community, and with key stakeholders.
Participants' Briefing:	Highlight the below to participants (in a clear, open, and non-threatening tone): Sexual Exploitation and abuse constitute a form of sexual misconduct perpetrated by UN/INGO/NGO and affiliated staff, volunteers, and contractors against community members receiving assistance and protection. Examples of such prohibited behavior includes: • Engaging in any sexual activity with minors or persons below 18 years – This includes grooming or betrothal to minors awaiting them to turn 18yrs. • Initiating or participating in a sexual or intimate relationship with persons receiving assistance and protection, • Exchanging goods, services, employment, or money for sex or sexual favors from beneficiaries or program participants, • Making any form of sexual advances towards beneficiaries or program participants, • Having sex with beneficiaries or program participants forcibly or using threats. • Sexual exploitation and abuse of persons receiving assistance and protection is a form of gross misconduct, and therefore grounds for termination of any implicated staff member. Additional measures include referral for criminal proceedings as per national laws. • The humanitarian community has a responsibility to protect all people receiving humanitarian assistance and protection from sexual exploitation and abuse, by ensuring that: 1. safe hiring practices are in place 2. all staff sign codes of conduct 3. all staff are trained and sensitized on the standards of conduct and SEA 4. All persons of concern are treated with dignity and respect 5. All persons of concern can safely report any incidents/allegations/concerns/suspensions of SEA against them or other community members, and receive immediate medical, psychosocial,

	material, and protection assistance. • All information you share will help us to identify additional gaps or risks of SEA that exist and enhance measures to prevent and respond to any SEA occurrences. • Ask if they have any questions about the process, before proceeding.
CONVERSATION GUIDE:	
Question 1: Risks	What, in your view, are the most significant risks in terms of SEA for this community?
Question 2: Mitigation	In addition to the measures outlined in the briefing, are there additional actions the humanitarian community/response could take to better protect persons of concern from these risks? If so, what more can be done?
Question 3: Reporting	In terms of available reporting mechanisms for SEA: • Do refugees know where and how would they report such incidents? • What reporting channels would refugees normally prefer to use? • Are there any obstacles to reporting? • What measures could humanitarian actors put in place to make reporting easier?
Question 4: Services	In terms of assistance for survivors of SEA: • -Do refugees in your community know where they can go for assistance following an incident of SEA? • -Where would refugees in your community prefer to go? (Prompt – medical, psychosocial, and legal support) • -Are there any obstacles to accessing assistance? • -What could the humanitarian community do to make it easier to access assistance?
Final Remarks:	Highlight the below to participants (in a clear, open, and non-judgmental tone): Importance of Reporting: • Any incident of SEA is serious. • Medical, psychosocial, material, and protection support are available for all victims of SEA if the victim is willing to receive such services. Investigations will be conducted by the agencies concerned. • A complainant, victim, or witness may report through: • Protection Desks • WFP/UNHCR Hotline • A trusted agency office/focal point • A trusted community member • A trusted agency staff member

	Reports will be processed and handled with confidentiality. The concerned agency will immediately link the victim or witness with relevant service providers, and initiate investigations if they are required to do so.
Conclusion:	a. Ask participants if they have any further comments or questions. b. Thank the participants for their contribution to the FGD and excuse yourself.

Data Tool 3: Activity Monitoring (at least 3 per location)

NAME AND LOCATION OF ACTIVITY SITE:		
CHECKLIST	OBSERVATION	OBSERVER'S COMMENTS
Activity sites and service points are identified in consultation with community members and are accessible to all affected persons including women, girls, and other at-risk groups (elderly persons, PWDs, ethnic minorities, etc.)	☐ Yes ☐ ☐ No	
Activities take place in an open and safe location (as per protection guidelines)	☐ Yes ☐ ☐ No	
Common facilities are constructed/set up in safe and accessible locations, identified in consultation with persons of concern	☐ Yes ☐ ☐ No	
There are separate lines/queues for male and female beneficiaries or there is active monitoring of queues at activity sites	☐ Yes ☐ ☐ No	
There are both male and female agency staff at activity sites and service points	☐ Yes ☐ ☐ No	
Case management services (protection (including CP/GBV response services, health centers, women's centers, child friendly spaces, and others) are carried out in a safe and confidential space that protects privacy	☐ Yes ☐ ☐ No	
All agency/humanitarian staff identify themselves with vests, badges, or otherwise (to be easily identifiable, especially for reporting issues and challenges)	☐ Yes ☐ ☐ No	
Are there unofficial aid workers (such as community volunteers/leaders) involved in the activities? Do they include female community volunteers?	☐ Yes ☐ ☐ No	
There are security guards (male and female) available at key service points or around facilities.	☐ Yes ☐ No	
There are confidential complaints mechanisms (confidential complaint box, hotline, email, or complaints desk)	☐ Yes ☐ No	
If challenges and issues arise activity implementation, they are handled by agency staff, and with the presence of a female employee if the person of concern is female	☐ Yes ☐ No	
There is clear information at the activity sites on:	☐ Yes	

1. The timing and content of distribution/activity 2. Aid is free 3. Reporting hotline or channel – how and where to provide feedback/complaints including SEA	☐ No ☐ Yes ☐ No ☐ Yes ☐ No	
Activities that involve direct contact with beneficiaries (such as in schools, health centers, shelter projects, food distribution, and others) are carried out by a team with a female staff member, and are closely supervised or monitored	☐ Yes ☐ No	
All humanitarian staff/workers in activity sites and service points have basic knowledge and understanding of Standards of Conduct and PSEA	☐ Yes ☐ No	